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Re: **Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care**

February 23, 2026

The following public comment responds to RIN 0955-AA13. I am a PhD-prepared Registered Nurse and tenured Professor at the University of Massachusetts Amherst. My clinical background includes emergency response and international disaster relief, patient navigation, gerontology and cancer care in both acute care and community-based settings. I am the founder and co-director of Health Tech for the People, a multidisciplinary think tank focused on accountable development and evaluation of health technologies. I have worked on A.I.-related research and policy for more than fifteen years.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?
   - The correct question should be what are the appropriate uses for A.I. in health care, and under what conditions/in what contexts? At present, there are a myriad of technologies purporting to be 'A.I', which has been deployed as a marketing term (Bender & Hanna, 2025). This confuses the conversation as we often lack universally adopted nomenclatures or conceptual precision.
   - Lack of engagement of 'end users' and those most impacted by the introduction or expansion of these technologies, especially patient and community groups marginalized within these systems, shift workers and laborers surveilled by these technologies, and care workers such as nurses, social workers, and educators (Walker et al., 2023).

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.
   - A.I. *in theory* has promised to transform health care. But too much A.I. *in practice* maintains a segregated, understaffed and unjust status quo, and a key driver of this disparity is *policy* that allows clinical care systems to make claims related to potential benefits of A.I. and similar technologies and act on them, without having to demonstrate those claims are actually true (Frizzell, 2023; Narayanan & Kapoor, 2024; National Nurses United, 2023; Pavuluri et al., 2024). Furthermore, mechanisms for redress of harms remain unclear as does legal culpability in the context of automated systems (and especially agentic A.I.).
   - Furthermore, recent reductions in protections for workers from impacts of automated surveillance and management systems (AWSM) pose a critical risk for the future of the

health care workforce (Garofalo et al., 2023; Hickok & Maslej, 2023; Office, 2024; Sinnreich & Gilbert, 2024).

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (*e.g.,* relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

- A.I. in theory has promised to transform health care. But too much A.I. in practice maintains a segregated, understaffed and unjust status quo, and a key driver of this disparity is policy.
- A.I. agents combine a chatbot mimicking human empathy with other health info. Unlike ChatGPT, they can initiate actions like assessments or referrals –even converse with other AI models - without being prompted. Hundreds are already for sale and nurses and other clinicians have been invited to train them. Under current Federal law these models are largely not allowed to diagnose or prescribe, though recently introduced legislation proposals such as HR 238 ("Healthy Technology Act") would redefine AI as a prescriber. This represents a paradigm shift that would fundamentally dismantle the patient-provider relationship as we know it and we do not yet have enough research to understand all the potential implications of this, and/or properly assess and regulate such a change.
- I'm practical about this: I live in a rural area, where it is often difficult to access care – especially primary care and specialty care. A.I. agents might remove some barriers to care access in some specific contexts. But Without STRONG community-driven policy and regulation with teeth, it will create new and long term problems such as even greater potentials for manipulation of patients (via targeted advertising of health services by behemoths such as Amazon and Meta), and longer term reduction and de-skilling of the health care workforce in areas where there are already severe shortages (Gerlich, 2025; Lee et al., 2025).
- We risk creating an even more segregated health care system where those who already have access to resources will be able to continue to access human and A.I.-supported care, while those who do not have adequate access will be forced to rely on remotely-managed and agentic forms of care (if they can access care at all).

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (*e.g.,* contracts, grants, cooperative agreements, and/or prize competitions)?
   - Prioritize grants to state and local level governance spaces which can help to shape the research agendas for those specific regions/contexts
   - Fund grassroots community organizations especially those representing constiutencies most likely to be impacted by deployment of A.I. systems (workers, patients, communities marginalized within decision-making spaces for health care and technologies).
   - Continue to provide grants to researchers and community advocates at public-serving academic/research institutions such as public universities, but do not require those grants

to “accelerate A.I. adoption”. We should have the capacity to look holistically at root causes and systems-level drivers of health inequities, and propose and evaluate solutions based on the approaches that best fit the priorities of the people. This may or may not involve ‘accelerating’ A.I. adoption in health care, depending on the nature of the problem definition and context.

5. How can HHS best support private sector activities (*e.g.,* accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

- HHS should consider all forms of this technology that may be required for the delivery of health care as digital public infrastructure, and fund and regulate them accordingly. Ceding this space to private industry may result in technological innovations, but those innovations will be designed to generate shareholder value rather than support health equity (Zucker, 2020).
- In calculating investments, HHS should consider the broader human and planetary costs of these technologies, many of which are so resource-intensive that they threaten to collapse ecosystems and reduce – rather than promote – public health in the longer term (Crawford, 2024; McDonald, 2024; Ricaurte, 2022; Schultz et al., 2025).
- Care workers like nurses currently have little access to information about how A.I. tools integrated into clinical care actually function. Although labels and ‘model cards’ have been suggested by groups like the Coalition for Health A.I. (CHAI: https://www.chai.org) , these are not mandated nor – in the rare cases they are made available – are they available to bedside workers in ways that would allow for meaningful use. More research, conducted in naturalistic settings, is needed to establish how these tools function and how best to allow both clinicians and patients to understand their functions, designs, possible benefits, and limitations.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

- A.I. tools that engage in ‘sous-veillance’ – pointing the camera back up at institutions and systems and holding them accountable, are more likely to generate effective actions to move us closer to health equity (Committee on the Future of Nursing 2020–2030 et al., 2021; Neumann & Bookey-Bassett, 2024).
- Without robust staffing regulation and efforts to ensure human-in-the-loop systems are actually true to their word, A.I. integration as a means of promoting efficiency may further exacerbate clinician “burn out” and health inequities. National Nurses United (NNU) has proposed a Nurse and Patient A.I. Bill of Rights which could help guide generation of a more robust, patient- and worker-centered approach to regulation and human rights: https://www.nationalnursesunited.org/sites/default/files/nnu/documents/0424_NursesPatients-BillOfRights_Principles-AI-Justice_flyer.pdf

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

- 'A.I.' should not be imposed on patients and workers without robust leadership of the groups most impacted by its use. Administrators should have less power in these decisions. Models for this such as community councils and Patient Advisory Boards could be expanded and robustly-funded to provide better governance and community control over A.I. technologies.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

- Enhanced interoperability would not accelerate movement towards greater health equity if data continue to be captured without meaningful consent or the ability of patients to take back their data. If we move to systems that involve some form of automated gatekeeping/triage (such as talking to an A.I. agent before seeing a human provider) we will create entire classes of patients who cannot safely access care due to the need to disclose characteristics or behaviors which have been increasingly criminalized and punished by our health systems. These range from demographic information such as information about sex or gender, to pregnancy status, to use of particular medications, to exposure to substances, to whether someone has legal U.S. citizenship (Eubanks, 2018).
- Increased interoperability and data-sharing under the current state of U.S. regulations and data-related civil rights (or lack thereof) is a direct threat to health equity due to the problem of mass surveillance and carceral care systems (Sinnreich & Gilbert, 2024).

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

- RESOURCES AND PLANETARY HEALTH: You can teach nurses and other clinicians to be become prompt engineers with tools like generative A.I. (genAI). But AI literacy must include considerations of resource, such as the fact much of genAI and agentic AI is powered by a growing number of resource-hungry data centers in people's backyards. For example, NY state had about ~150 data centers at my last count. Last year U.S. data centers consumed 200 tera-watts of electricity – enough to power all of Thailand for a year - expected to triple in the next 3 years. Its estimated data center spread in Virginia costs citizens an extra $37/month. They can be a source of environmental racism- poisoning communities with emissions. This is a nursing and public health problem. The handful of proposed "zero water" centers won't be online for at least 2 more years, by which point it is estimated they'll have drained another trillion gallons of water, much of it in places where farmers' fields are fallow, tap water has dried up, and ecosystems have collapsed. "Green AI" pilots are built on companies' knowledge that by 2030 in many areas the water will already be gone. Will AI adoption matter if our communities don't have water & reliable electricity? We demand policies to immediately halt A.I.'s secretive & profligate consumption of non-renewable planetary resources, or it will cost us access to clean air and water, a reliable grid and a livable planet.

- PATIENT AND WORKER PRIVACY: Existing legislation expects individual consumers to know how to opt into privacy, & its impossible for anyone to fully opt out of having your data collected & sold. The EU passed GDPR legislation- the "right to be forgotten law- which provides broad protections against invasive surveillance. But US industry has fought GDPR & every attempt to rein in their power. When the ACA became law it required health records be sent to a national database. Last year I sat on a HITAC committee for a Federal Rules Task Force tasked with recommending how to do that. A national database of all EHR data. Only certain types of reproductive health data can be held back. I fought for that definition of repro health data to be as BROAD as possible, so communities which tend to be targeted by surveillant A.I. and its carceral consequences could better protect ourselves. Why? Because if you're LGBTQ+, or an immigrant, disabled, pregnant, in recovery or seeking or providing any of the care increasingly criminalized, you know how easily such data can be weaponized, especially with A.I. And since health systems violate federal law for not sharing these data, we need to teach clinicians how to chart defensively –to think critically about what data are captured & how patients can prevent unwanted surveillance. Which also means rethinking policies around ambient listening in the clinic.
- CLINICIAN AUTONOMY: Surveillant A.I. impacts clinician well-being & autonomy. Worker surveillance, similar to that applied to Amazon warehouses & gig drivers, may help "optimize" flexible staffing but without just policy, AI-driven hyper-efficiencies can drive workers into the ground. AI has been rolled out in more than 80% of U.S. hospitals. A recent poll of over 2000 nurses revealed half worked at clinics that use AI to determine patient acuity, 69% of which said their expert clinical assessments don't match those scores & often they can't do anything about it (National Nurses United, n.d.). A majority didn't trust their employers to implement AI safely, which makes sense given what we saw during the pandemic. As a nurse & patient safety issue, the NNU has called for a Nurses' & Patients' A.I. Bill of Rights – we should operationalize this through policy.
- DATA WEAPONIZATION: With A.I. agents, every interaction is recorded. Historically, human health care workers have resisted unjust policies like bans on gender-affirming care or abortion. But with so much care politicized and criminalized, automated recording AND reporting has consequences. For example, "voice biomarkers" are being trained to detect illness & cognitive decline in people's audio recordings. Such tech isn't necessarily accurate – but that doesn't mean it won't be weaponized by law enforcement, insurance companies & other groups (Fagherazzi et al., 2021).
- To what extent does deployment of A.I. tools enhance accessibility and health equity for patients with disabilities? And to what extent do they amplify technoableism? (Shew, 2024)

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

- When AI is introduced, is there any disclosure of how it works across intersectional groups, like a model card? How can we make information about A.I., how it functions, its performance compared to alternatives or usual care, and impacts – proximal and distal (like resource consumption) legible and transparent to patients, care workers, and other decision makers?

- A.I. supposed to "free up" nurses and other clinicians to provide more humanizing care. But in the absence of robust safe staffing legislation, do you know if it is actually doing that? Research by Frennert, Elish & others shows its a VERY MIXED bag. Don't just ask but MEASURE: As AI is implemented, is time with patients actually increasing? Do individual patients get more time with their clinicians? How about clinician well-being – is their work made easier or are they simply asked to take on more (with fewer human colleagues present)?
- Is the invisible labor and repair work associated with A.I. roll out and maintenance accounted for in clinical workflows and resource allocation? (Dillard-Wright, 2019; Elish & Watkins, 2020; Frennert et al., 2023)
- As agentic A.I. is rolled out – what's happening in primary care, acute care, aging care, disability care? For what patient groups/contexts is such technology actually beneficial (if at all), and for whom is it a barrier to care/health equity? What are the systems-level changes that are needed to ensure actual health equity (beyond implementation of A.I.) and how can we achieve them?
- What does precepting of clinicians and care workers look like in an A.I. era and are there enough? Are there adequate safeguards to assess clinical performance and on-going development, especially if A.I. adoption leads to reductions in health care staffing?
- <u>Translate Care Work.</u> The public needs to understand the value of care work. Presently care is mostly imagined as a billable commodity, rather relational praxis. Do institutional policies ensure technologies support anti-racist, equitable and *consentful* care and education? Does such care translate to students & employees? How can this best be measured and assessed in the short- and long-term?

a. Are there published findings about the impact of adopted AI tools and their use clinical care?

Yes, please see bibliography below.

b. How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

Tech says A.I. is revolutionary and whether you like it or not, you need to get on-board. But revolutions are about challenging power. In our current policy landscape –A.I. is less about revolution and more about consolidating power & resources in the hands of a very few; entrenching an unjust, chronically understaffed, undertreated and at times, profoundly carceral status quo.

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